

Heart Failure Discharge and Transitional Care Protocol

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Owner	Cardiology Service Line and Care Transitions		
Approved by	Clinical Quality Committee		
Applies to	Riverside Memorial Hospital, Lakeview Community Hospital, Eastgate Regional Hospital and affiliated clinics		

1. Purpose and scope

This protocol sets out how Riverside Health Network prepares adult patients with heart failure (HF) for discharge and how it supports them in the 90 days afterwards. Its goal is to cut avoidable 30-day readmissions by making the first week after discharge predictable: every eligible patient leaves with a scheduled follow-up visit, a working home scale, a reconciled medication list and a named nurse who will call within 48 hours.

The protocol applies to every inpatient admission whose primary diagnosis is heart failure (ICD-10 codes I50.x, for example I50.9 heart failure unspecified, I50.22 chronic systolic heart failure, I50.23 acute on chronic systolic heart failure and I50.32 chronic diastolic heart failure) at Riverside Memorial Hospital (F01), Lakeview Community Hospital (F02) and Eastgate Regional Hospital (F03). Patients with HF as a secondary diagnosis follow the unit's general discharge process, with this protocol's education items as a recommended addition.

2. Definitions

Index discharge. The discharge that ends an inpatient HF admission. Discharges that end in death or in transfer to hospice are not index discharges for quality reporting.

Eligible for the 7-day visit. An index discharge to home or to home health. Patients discharged to a skilled nursing facility, to inpatient rehabilitation or against medical advice follow section 6.

Completed follow-up visit. A kept outpatient encounter, in person or by telehealth, with a physician, nurse practitioner or physician assistant at a Riverside clinic or hospital outpatient department. A phone call from the Care Transitions nurse does not count as a visit.

30-day readmission. Any later inpatient admission of the same patient that begins within 30 days of the index discharge, whatever its cause.

3. Discharge readiness criteria

The discharging physician confirms and documents each criterion before the discharge order is signed. The Care Transitions nurse completes the discharge checklist (form RHN-F-0231) in parallel; a scanned copy is filed in the chart.

- Weight is at or within 3 lb of the documented dry weight, and the discharge weight is recorded on the checklist.
- Oral diuretic regimen has been stable for at least 24 hours with no intravenous diuretic in that period.
- Potassium and creatinine from the last 24 hours have been reviewed.
- Resting heart rate and blood pressure are within the targets the cardiology team has written in the plan of care.

- The patient or caregiver has a working home scale, or one has been supplied by the unit before the patient leaves.
- A follow-up visit within 7 days has been booked and the date is written on the checklist and the after-visit summary.

Note: If any criterion cannot be met, the physician documents why and the Care Transitions manager is told the same day. A discharge is not delayed for a missing appointment alone: the nurse books the visit before the patient leaves the unit or, at the latest, within one business day of discharge.

4. Medication reconciliation and patient education

4.1 Medication reconciliation

A pharmacist or trained nurse reconciles the home medication list against the discharge list. Changes to diuretics and to guideline-directed medical therapy (beta-blocker, ARNI or ACE inhibitor, mineralocorticoid antagonist and SGLT2 inhibitor) are explained in plain language and printed on the after-visit summary. The reconciled list is sent to the receiving clinic with the discharge summary within 24 hours.

4.2 Education using teach-back

The nurse teaches and checks understanding, using the patient's own words, of five points: daily morning weights; the 2 lb in one day or 5 lb in one week rule for calling the clinic; a sodium limit of 2,000 mg a day; how each medicine works and when not to take it; and the symptoms that mean call 911. Where a caregiver helps at home, the teach-back is repeated with the caregiver. A teach-back that was not completed is recorded as an open item on the checklist and passed to the 48-hour call.

5. Post-discharge follow-up schedule

Every eligible patient receives the schedule below. The unit books the 7-day visit and the 14-day laboratory order before discharge; Care Transitions owns the schedule from the day of discharge.

Timeframe	Required action	Owner	Notes
Within 48 hours of discharge	Nurse phone call: symptoms, daily weight, medicines in hand, open checklist items	Care Transitions RN	Up to 3 attempts in 48 hours; every attempt is documented
Within 7 calendar days	Follow-up visit at an HF clinic (Pine Valley Heart Failure Clinic, F10, or Oakridge Cardiology Clinic, F11), the patient's primary care clinic, or by telehealth	Receiving clinic	Booked before discharge; a kept visit is required, a call does not count
Within 14 days	Basic metabolic panel (potassium, creatinine) after any change to a diuretic, ARNI, ACE inhibitor or MRA dose	Ordering clinic	Result reviewed within 2 business days
Within 30 days	Cardiology or HF clinic review: volume status and titration of guideline-directed therapy	Cardiology	Repeat the checklist's education items if the patient has been readmitted
At 90 days	Functional status, medication adherence and need for repeat echocardiogram	Primary care or HF clinic	Closes the transitional care episode

5.1 The 7-day visit

The visit must take place within 7 calendar days of the discharge date. The clinic confirms the appointment by phone 2 days before it. Transport is arranged by Care Transitions for any patient who has no ride. A visit on day 7 counts; a visit on day 8 or later is recorded as late and still prompts the 14-day actions.

5.2 Missed or unbooked visits

If the 7-day visit has not been completed by the end of day 7, the Care Transitions nurse:

1. calls the patient the same day and reschedules the visit within 2 business days;
2. records the reason (no transport, patient declined, clinic capacity, other);
3. escalates to the Care Transitions manager if the visit is still not completed by day 10; and
4. lists the patient in the monthly missed follow-up report that goes to the Clinical Quality Committee.

6. Other discharge destinations

Discharge destination	What this protocol requires
Home or home health	Full schedule in section 5; counted in the 7-day completion measure
Skilled nursing facility or inpatient rehabilitation	Discharge summary and medication list to the facility within 24 hours; facility physician reviews within 72 hours of arrival; not counted in the 7-day measure
Against medical advice	Care Transitions calls within 24 hours and offers a visit within 7 days; not counted in the 7-day measure
Hospice	Palliative plan of care; excluded from every measure in section 8

A patient readmitted within 30 days of an HF discharge is reviewed by the unit's nurse manager and the Care Transitions manager within 5 business days. The review asks which schedule steps were missed and whether the patient could have been supported differently.

7. Roles and responsibilities

Role	Responsibility
Discharging physician	Confirms the readiness criteria in section 3 and writes the plan of care
Unit nurse	Completes the checklist, performs teach-back, confirms the home scale, books the 7-day visit
Pharmacist	Reconciles medicines and reviews the discharge list
Care Transitions RN	Makes the 48-hour call, owns the schedule, reschedules missed visits, reports open items
Receiving clinic	Holds capacity for HF visits within 7 days and reports no-shows the same day
Care Transitions manager	Reviews escalations and readmissions; reports monthly to the Clinical Quality Committee

8. Quality measures and targets

Measure	Definition	Target	Reported
7-day follow-up completion	Eligible HF discharges with a completed outpatient visit within 7 calendar days, divided by eligible HF discharges	85% or higher	Monthly, by hospital
30-day HF readmission rate	Index HF discharges followed by any inpatient admission within 30 days, divided by index HF discharges	20.0% or lower	Quarterly, by hospital
48-hour call completion	Eligible discharges reached by phone within 48 hours	90% or higher	Monthly
Teach-back documented	Discharges with teach-back recorded for patient and, where applicable, caregiver	95% or higher	Monthly

9. Version history

Version	Date	Change
4.2	2026-01-15	Added the 48-hour call, the 7-day visit requirement for home and home health discharges, and the missed-visit escalation in section 5.2
4.1	2024-11-01	Added teach-back with caregivers and the home scale criterion
4.0	2023-06-12	Moved ownership to Care Transitions and Cardiology jointly

Riverside Health Network is a fictional organization. This document is synthetic and is not clinical guidance.